

Siteman Cancer Center · Washington University School of Medicine

An orbit in support of *pancreatic cancer surgery* patients and their families.

A controlled study of patients with pancreatic ductal adenocarcinoma consented to surgery, and the family members who care for them. Participants were issued CareOrbit and otherwise treated identically to a matched control group.

+65%

UNDERSTANDING OF PLAN AND SIDE EFFECTS

Improvement in patient understanding of the disease, treatment specifics, and pre- and post-treatment plans.

+42% among family members

87%

ABILITY TO NAVIGATE THE CARE JOURNEY

Patients improving in working with their care team, appointments, required actions, and follow-ups.

77% of family members

-53%

CALLS TO THE OFFICE, PRE-OP

Fewer inbound calls before surgery, indicating patients and families arrived more prepared.

-39% post-op

-41%

READMISSION RATE

Reduction in readmissions against the matched control group.

+22%

PATIENT SATISFACTION

Improvement in patient HCAHPS and SF-36 satisfaction scores.

+38% family satisfaction

96%

USE OF SUPPORT RESOURCES

Patients and families who reported linking to additional support and educational resources from their orbits.

90%

ACTIVATION RATE

Of all users issued an orbit, activated it, in under 4 days on average from invitation.

107

PAGE VIEWS PER USER

Average individual page views per patient and family member in their initial period of usage.

~125

PARTICIPANTS

50 patients plus roughly 75 family members, all issued orbits, Whipple or RAMPS resection for PDAC.

How care teams responded

Surgeons, nurses, and managers were interviewed throughout the study to capture what they observed as orbits ran with patients and families. A representative sample:

It helps evolve the quality of our conversations, leading to understanding happening earlier for everyone who could be part of the patient's improvement.

ON COMMUNICATION

We use it to open up the dialogue, making them more aware of key topics more quickly.

ON COMMUNICATION

They get things explained to them, not just pushed on them.

ON COMMUNICATION

They are nervous and confused and it helps them see what they face in ways they are comfortable around.

ON ANXIETY AND CONFUSION

When I show them the orbit, their eyes light up and they are so excited they are going to have access to it.

ON QUALITY OF EXPERIENCE

They love seeing their team in the orbit. This extra step, to them, means we really care and we're all on the same page with them.

ON QUALITY OF EXPERIENCE

It was hard to deny the control group orbits for use during our study. We knew just how much an orbit could help them right now.

ON THE STUDY ITSELF

I'm a nurse and I wish my own Dad had an orbit when he went through his procedure.

ON THE STUDY ITSELF

Study details

STUDY AUTHOR

Led by Dr. Ryan C. Fields, chief of the Section of Surgical Oncology at Washington University School of Medicine, who treats patients with liver tumors, pancreatic cancer, gastric cancer, sarcoma, and melanoma at Siteman Cancer Center, St. Louis. Dr. Fields runs an NIH-funded translational research laboratory focused on patients at high risk of cancer recurrence.

PARTICIPANTS AND CRITERIA

Patients diagnosed with pancreatic ductal adenocarcinoma who had consented to surgery (Whipple or RAMPS) as part of their treatment plan. Results reflect the initial group of approximately 125 users: 50 patients plus roughly 75 family members, all issued orbits.

METHOD

Participants were issued CareOrbit but otherwise treated identically to a matched non-CareOrbit control group. Measures such as navigation of the journey and understanding of treatment and side effects were obtained via validated surveys conducted as personal interviews with patients and family members.

WHAT CAN BE SHARED

This is the extent of what can be shared publicly prior to publication; some details of the validated survey remain confidential until then. Siteman Cancer Center is currently expanding use of orbits, with multiple orbits now in development.

For study specifications and specifics, contact support@careorbit.com. Dr. Fields has made himself available for discussion with interested professional parties, including competitive health systems.

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